The white army



Mentor -
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Presented by
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Patient particulars

- Name – Mr N
- Age – 54 years
- Gender – male
- Address – Malleshwaram
- Occupation – tailor
- Date of admission – 7th September 2021
- Date of examination – 16th September 2021

Chief Complaints

- Breathlessness – since 1 year (exacerbated 3 days back)
- Cough – 3 months
- Fever – 3 days

History of presenting illness

- Patient was apparently normal 1 year back when he noticed breathlessness on climbing 1 floor of stairs and on walking uphill. It was insidious in onset and relieved with rest. It progressed 3 months back to cause breathlessness with hurrying on level ground and impaired his daily activity, for which he used to visit the hospital where he would received nebulisation. From the past 1 month patient has been on home nebulisation. There was further worsening three days back to cause breathlessness on walking for about 100m. Not associated with wheeze. There was no history of awakening in the night or breathlessness on lying down and no diurnal variations.

- Patient complains of cough from 3 months, insidious in onset and associated with sputum. Sputum was scanty in amount, white in colour, mucoid in consistency non-foul-smelling not blood tinged. No diurnal variation, no associated postural variation.

- There was fever since three days continuous and Not associated with chills and rigour and relieved with medication.

- Currently at the time of examination his symptoms have markedly subsided.

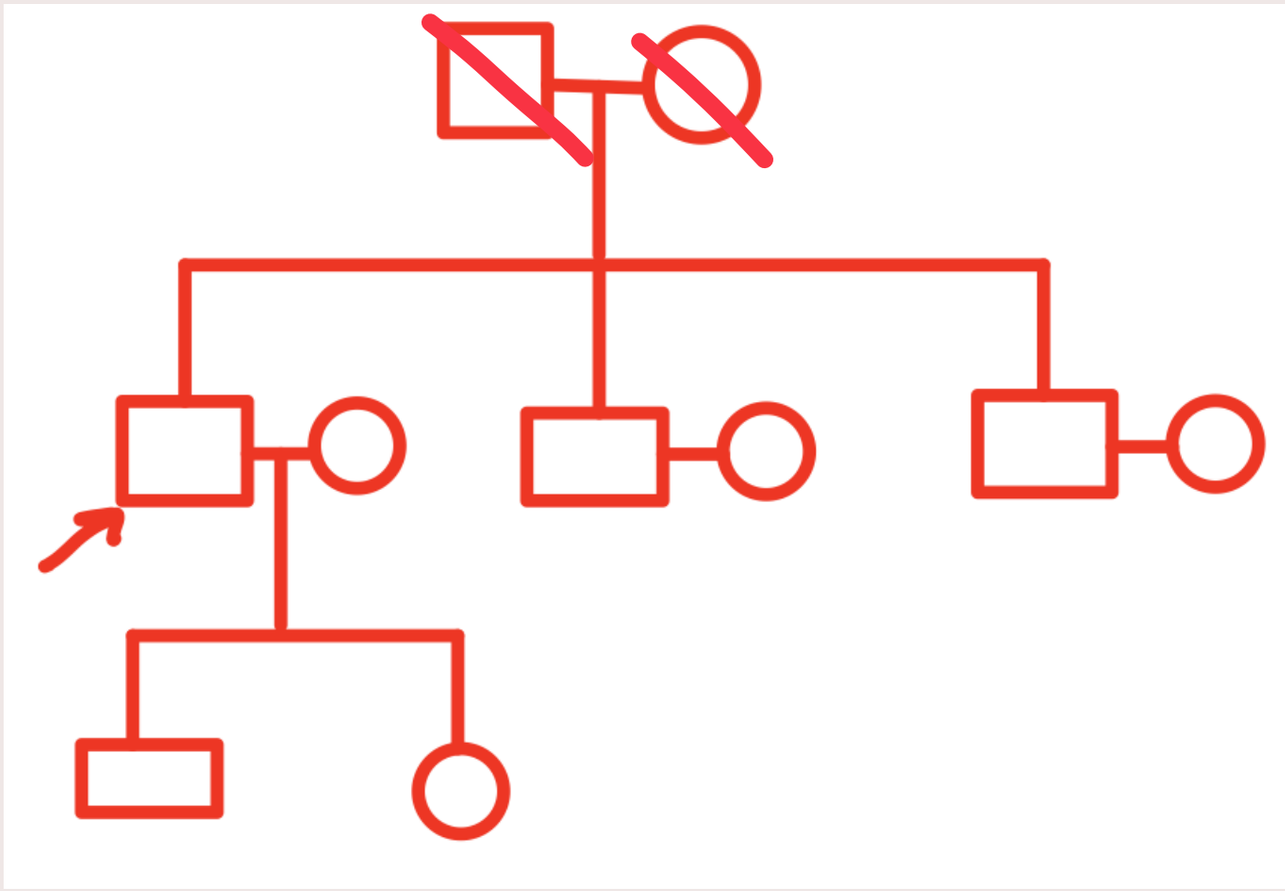
- No history of wheeze
- No history of chest pain, palpitation, syncope
- No history of upper respiratory tract infections
- No history of night sweats, weight loss and loss of appetite
- No history of easy fatiguability
- No history of bilateral lower limb swelling
- No history of epigastric pain
- No known allergies
- No history of morning headache.
- No history of contact with COVID patient.

Past history

- Not a k/c/o Hypertension, diabetes mellitus, tuberculosis, asthma or heart diseases.
- No history of previous hospitalisations.

Family history

- No similar complaints in the family



Personal history

- Diet – mixed
- Appetite – normal
- Sleep – adequate
- Bowel and bladder – regular
- Smoker since 30 years - 1 pack cigarette per day (30 pack years)
- Smoking index 600

Summary

- Middle aged male patient who is a chronic smoker with no known comorbidities presented with worsening breathlessness (grade 1 to grade 3) and fever since 3 days. Cough with expectoration since 3 months.

- From history It appears to be a respiratory pathology. probably acute exacerbation of obstructive disease.

- D/d

- Interstitial lung diseases

- Hypersensitivity pneumonitis

General physical examination

- Here is a middle aged male , moderately built and poorly nourished, conscious, co-operative and alert. Well oriented to time, place and person.
- Height 160 cm, weight 52 kg, BMI - 20.3 kg/m2
- Vitals : Pulse - 73 beats /min, normal in rhythm, volume, no radio – radial delay , radio – femoral delay, all peripheral pulses felt , no vessel wall thickening.
- Respiratory rate - 20 cycles/min, thoracoabdominal type .
- Blood pressure – 115/80 mm Hg measured in right upper arm.
- Temperature – 37.6 *celsius (measured in axilla)

- Grade 2 clubbing seen
- No pallor, cyanosis, icterus, edema, or lymphadenopathy
- Fine tremors present
- No external markers of TB, malignancy
- Spine normal

Respiratory system examination

Upper Respiratory Tract:

- Nasal cavity: normal

- Oral cavity:oral thrush present.

- Pharynx appears normal

Lower respiratory tract – inspection

Chest is barrel shape and bilaterally symmetrical

- Trachea appears to be central

- Apex beat cannot be visualised

- Use of accessory muscles of respiration absent

- No dilated veins, scars or sinuses visible on chest wall

- Kyphoscoliosis not present

  Slight drooping of left shoulder present

- No indrawing of intercostal spaces

- Chest movements

Area	Right	Left
Upper anterior chest	Diminished	Diminished
Lower anterior chest	Diminished	Diminished
Upper posterior chest	Diminished	Diminished
Lower posterior chest	Diminished	Diminished

- Movements are bilaterally symmetrical

Palpation

Inspectory findings are confirmed
•Trachea is central in position
•Apex beat not palpable
•No local raise of temperature
•Movements of chest: symmetrical

Area	Right	Left
Supraclavicular		
Infraclavicular	Symmetrical on both sides and	
Mammary	appears to be diminished	
Suprascapular		
Infrascapular		

- Tactile vocal fremitus

Areas	Right	Left
Supraclavicular		
Clavicular		
Infraclavicular		
Mammary	Symmetrical on both sides	
Axillary	and appears to be	
Infra-axillary	Decreased	
Suprascapular		
Interscapular		
Infrascapular		

- Measurements

T diameter		28.5 cm
AP diameter		26 cm
Chest circumference	Expiration	84 cm
	Inspiration	85 cm
Right hemithorax	Expiration	42 cm
	Inspiration	42.5 cm
Left hemithorax	Expiration	42 cm
	Inspiration	42.5 cm
Chest expansion	Right hemithorax	0.5 cm
	Left hemithorax	0.5 cm
	Total	1 cm

Percussion

Areas	Right	Left
Supraclavicular		
Clavicular		
Infraclavicular		
Mammary	Symmetrical on both sides and	
Axillary	appears to be hyper resonant	
Infra-axillary		
Suprascapular		
Interscapular		
Infrascapular		

Cardiac dullness obliterated

Auscultation

Breath sounds

Areas	Right	Left
Supraclavicular		
Clavicular		
Infraclavicular	Symmetrical on both sides and appears to be diminished vesicular breath sounds.	
Mammary		
Axillary		
Infra-axillary		
Suprascapular	No added , adventitious sounds	
Interscapular		
Infrascapular		

- Vocal resonance
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Areas	Right	Left
Supraclavicular		
Clavicular		
Infraclavicular		
Mammary	Symmetrical on both sides	
Axillary	and appears to be	
Infra-axillary	Decreased	
Suprascapular		
Interscapular		
Infrascapular		

Other system examination

- Cardiovascular system S1 heard normally, p2 loud, No murmurs heard No additional sound heard, JVP not elevated
- CNS Higher mental functions normal No flapping tremors / altered sensorium No focal deficits
- Per abdomen : Soft, non tender abdomen. No organomegaly or ascites. Normal bowel sounds.

Summary

- Middle aged male patient who is a chronic smoker with no known comorbidities presented with worsening breathlessness (grade 1 to grade 3) and fever since 3 days. Cough with expectoration since 3 months.
- On examination he was undernourished, he had grade 2 clubbing. There was barrel shaped chest, symmetrical decrease in chest movements, hyper resonant on percussion with decreased intensity of breath sounds. There was loud P2

Provisional diagnosis

- Acute Exacerbation of Bilateral obstructive lung diseases, emphysema predominant secondary to chronic smoking without evidence of cor pulmonale or respiratory failure.
- Exacerbation is probably due to viral/ atypical pneumonia
- Appears to be mixed ILD and COPD (emphysema)
- D/d -
- Interstitial lung diseases
- Hypersensitive pneumonitis
- Asthma
- Tb
- lung malignancies

Investigations

- COPD questionnaire
- Complete blood counts
- Eosinophil count and serum IgE levels
- Sputum culture and sensitivity
- Chest X ray, PA and lateral view
- Spirometry
- Body plethysmograph
- HRCT

Treatment

- Stop smoking
- Treat according to category of patient
- Antibiotics
- Oxygen


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Thank you